Project memo: APAC healthcare acquisition

To: Engagement partner From: Engagement team Date: 31 October 2026 Status: Draft for partner review

Decision question: Should we proceed with the acquisition, and what must we validate before committing to the investment?

1. Recommendation

Do not approve the full investment yet. Approve progression to confirmatory diligence, with a signing decision gated on the eight validation items in section 7. The sector case is credible: APAC healthcare is ageing, demand is rising, and dealmaking is active. But the evidence we can defend today supports the sector, not the deal. Three gaps stop us recommending a commitment.

First, the target's segment is unspecified (provider, pharma, medtech, diagnostics or health-tech), and the main risks differ sharply by segment. Government price-setting that cut some Chinese device prices by roughly 90% in one studied category (E11, E12) matters a great deal to a device maker and much less to a hospital operator. Second, medical cost inflation is running at record levels (E7), so the target's ability to pass costs through is the key financial question and has not been tested. Third, stakeholder claims have not yet been reconciled against evidence (section 6).

A competitive deal market (E4, E5) also suggests pricing pressure on the buyer. That is our inference rather than a sourced finding, and valuation work must test it against actual comparable transactions.

2. How to read the evidence

Every external claim below carries a reference to the evidence register in the appendix, shown as: ID (source date; credibility). Credibility is our rating. High means peer-reviewed or primary data with a disclosed method. Medium-High is a reputable publisher with a disclosed method. Medium is credible but secondary or partly opaque. Low is vendor, aggregator or blog content with an undisclosed method. 'n/c' means the exact publication date was not confirmed.

Limitation to flag before this reaches the committee: in this working environment we could read search-result summaries but could not open the publishers' pages (the PwC and Bain sites were blocked). Every figure should therefore be checked against the full source and its publication date before the deck is issued. Claims rated Low are used for direction only, and E19 (the '70-90% of deals fail' figure) is deliberately not relied on.

3. Sector overview

Market size forecasts exist, but they are weak evidence. Commercial vendors put APAC healthcare at about USD 2.85tn in 2025, growing near 8% a year to 2035 (E1: 2026; Low). Segment forecasts are higher for digital and home care: about 21.8% a year for smart healthcare and 12.5% for home healthcare (E2, E3: n/c; Low). These vendors define 'healthcare' differently and disclose little method, so we use them only to say that growth is expected to be positive and faster in digital and home care. They should not be quoted as a sized addressable market. Sizing needs to be rebuilt bottom-up for the target's actual segment and countries.

Dealmaking is the better-evidenced signal. Global health-industries deal value rose 46% in 2025 while volumes fell 5%, and APAC had the largest regional volume rise at 12%, helped by a 53% jump in China (E4: 2026 outlook; Medium-High, with the caveat that PwC is the publisher). APAC accounted for roughly 16% of global healthcare private equity deal value in 2024-25 (E5: 2026; Medium-High), and PE buyouts strengthened again in H1 2026 (E6: H1 2026; Medium). Practitioner commentary reports that India has the highest APAC deal count and that joint ventures are common where foreign ownership is restricted (E20: 2026; Low-Medium), which needs confirmation from local counsel.

4. Competitor comparison

A like-for-like comparison is not possible until the target's segment is known. The table covers listed regional hospital operators, the most likely competitor set if the target is a provider. All figures come from low-credibility aggregators and use different currencies and years, so they show relative scale only. They must be replaced with figures from filed annual reports before use.

Operator	Footprint (per source)	Scale indicator (per source)	Evidence
IHH Healthcare	Malaysia-based; 89 hospitals in 10 countries; owns Fortis	Revenue not retrieved	E17 (May 2026 (from URL); Low)
Apollo Hospitals	India; 73 hospitals, 8,000+ operational beds	FY25 revenue Rs 21,794 crore; hospital, pharmacy and diagnostics mix	E16 (FY25 data; Low)
Fortis Healthcare	India; roughly 5,500-5,800 beds	FY25 revenue Rs 7,783 crore	E16 (FY25 data; Low)
Ramsay Health Care	Mainly Australia, UK and Europe	Group revenue A$17.8bn (2025); mostly outside APAC emerging markets	E17 (May 2026 (from URL); Low)
Raffles Medical Group	Singapore	Revenue not retrieved; PitchBook lists Fortis, KPJ, Manipal, Apollo and IHH as competitors	Not in register (PitchBook snippet; to verify)

Two points follow. The regional competitor set is already consolidated around a few multi-country and national groups, and IHH's ownership of Fortis shows that scale players are active buyers, so the acquirer may be bidding against them. Whether the target is a niche, a platform or a bolt-on cannot be judged until its segment and market share are known. For a pharma, medtech, diagnostics or health-tech target the peer set and metrics will differ and this section should be rebuilt.

5. Major trends, opportunities and risks

Major trends

Trend	What the evidence says	Implication for the deal	Evidence
Record medical cost inflation	Asia medical trend forecast at 12.5% for 2026, the sixth consecutive double-digit year; Indonesia 17.8%, Philippines 16%, India 9.9%. Aon sees APAC easing from 11.3% to 9.7% in 2027.	Margins depend on repricing or cost control. Test the target's contracts and payer mix.	E7 (2026 edition; Medium-High); E8 (2026; Medium)
Active, increasingly PE-led dealmaking	APAC volume up 12% in 2025; APAC about 16% of global healthcare PE value; PE strengthening in H1 2026.	Competition for assets; risk of paying a full price.	E4 (2026 outlook; Medium-High); E5 (2026; Medium-High); E6 (H1 2026; Medium)
Government price-setting	China's VBP guarantees hospitals volume in return for steep price cuts; stent prices fell 92.67% in one Tianjin study; one author calls VBP permanent.	Critical for devices and drugs; much less so for providers. Depends on segment.	E11 (n/c; High); E12 (n/c; Medium)
Tighter and fragmented regulation	Device rules tightening in 2026; registration is country-by-country; timelines of 12-24 months reported for China and Japan.	Slower market entry and higher compliance cost than synergy models often assume.	E9 (2026; Medium); E10 (2026; Low)
Rapid AI adoption, weak foundations	81% report generative AI use, but 67% cite cost as the main barrier and only 35% have dedicated AI teams; fragmented EHRs and privacy rules limit scale.	AI upside is real but depends on data infrastructure the target may lack.	E13 (24 Aug 2026; Medium); E14 (2026; Medium-High); E15 (2026; Medium)

Opportunities

Opportunity	Basis	Condition to realise	Evidence
Demand growth in digital and home care	Vendor forecasts of 12-22% annual growth	Forecasts are Low credibility; size the segment bottom-up first	E2 (n/c; Low); E3 (n/c; Low)

Opportunity	Basis	Condition to realise	Evidence
Platform and roll-up plays in fragmented markets	Investor focus on India hospitals, diagnostics and specialty networks	Needs local counsel on ownership limits and a JV option	E20 (2026; Low-Medium)
AI-enabled productivity	75% of providers expect gains from agentic AI	Requires clean, interoperable data and an internal AI team	E13 (24 Aug 2026; Medium); E14 (2026; Medium-High)
Regulatory reliance pathways	Bilateral reliance schemes launched in ASEAN in 2025-26	Reduce but do not remove local submission needs	E10 (2026; Low)

Risks

Risk	Why it matters	Evidence
Pricing and reimbursement	State procurement can remove most of the price premium on affected products. Exposure depends on the target's mix.	E11 (n/c; High); E12 (n/c; Medium)
Cost inflation and pass-through	Insurer-projected costs are rising far faster than general inflation; if payers resist, margins compress.	E7 (2026 edition; Medium-High); E8 (2026; Medium)
Regulatory approval and licensing	Country-by-country approvals, local-language dossiers, and ownership restrictions in some markets.	E9 (2026; Medium); E10 (2026; Low); E20 (2026; Low-Medium)
Data, cyber and technology integration	Data localisation, sovereignty and incompatible EHRs complicate a multinational integration. Evidence on specific country rules outside China is thin.	E15 (2026; Medium); E14 (2026; Medium-High)
Overpayment and integration	Diligence that misses sector-specific costs can create unplanned recurring costs (one EY example above USD 30m a year). The often-quoted '70-90% fail' figure is not reliable.	E18 (n/c; Medium-High); E19 (n/c; Low)

6. Reconciling stakeholder perspectives

We do not yet have the interview content. The table gives the evidence test to apply to each function's claims. The second column holds hypotheses about where conflicts typically arise, not findings from the interviews. Each real claim should be logged, matched to evidence, and marked supported, contradicted or untested.

Function	Claim type to test (hypothesis)	Evidence test	Status
Business leaders	Market growth and cross-selling synergies	Rebuild segment sizing bottom-up (E1 to E3 are not usable as is); check the synergy case line by line against comparable deals (E18)	Awaiting interview input
Technology	Integration speed and AI upside	Review EHR landscape, data localisation constraints and AI team capacity (E13 to E15)	Awaiting interview input
Finance	Valuation, margin and cost assumptions	Test against the 2026-27 medical trend range (E7, E8) and actual comparable transaction multiples from primary deal data	Awaiting interview input
Operations	Capacity, workforce and licensing	Check licences and ownership structure with local counsel (E9, E10, E20); verify staffing data directly	Awaiting interview input

7. What must be validated before committing

Each gate has an evidence requirement and a stop condition. The triggers are proposals for the committee to set, not findings.

#	Validate	Evidence required	Stop / re-price trigger (proposed)
1	Segment and country exposure to price-setting	Revenue by product, payer and country mapped to procurement or price-control schemes	Material share of revenue in categories under volume-based procurement or price caps, with no mitigation
2	Cost pass-through	Payer contracts, repricing history, margin sensitivity to 10-12% cost growth	Plan only works if costs are passed through in full

#	Validate	Evidence required	Stop / re-price trigger (proposed)
3	Licences and ownership	Local counsel opinion on foreign ownership, change-of-control and JV need	Structure cannot deliver control or the required licences
4	Regulatory pipeline	Product-by-product approval status and timelines from regulators' own sources	Revenue plan relies on approvals outside the realistic window
5	Data, cyber and IT	Data-flow mapping, localisation review, cyber assessment, EHR inventory	Unremediable localisation conflict or major unresolved breach
6	Workforce and operations	Clinician headcount, attrition, compensation harmonisation cost	Unbudgeted recurring costs of the kind in E18
7	Synergies and integration	Bottom-up synergy case with named owners; integration and culture plan started during diligence	Deal only clears the price hurdle if synergies are assumed at the top of the range
8	Valuation and competition	Comparable transactions from primary deal data; view on competing bidders	Price implies returns only achievable with full synergies

Appendix: Evidence register

All entries were read as search-result summaries; full publisher pages were not opened. Verification status for every entry: to confirm against the full source. Credibility is the team's assessment.

ID	Claim	Source and URL	Source date	Type	Credibility	Rationale
E1	APAC healthcare market USD 2.85tn (2025) rising to USD 6.07tn (2035), CAGR 7.89%	SNS Insider, APAC Healthcare Market 2026-2035 https://www.snsinsider.com/reports/apac-healthcare-market-11013	2026 (day n/c)	Commercial market research	Low	Paid-report marketing page; 'healthcare' definition undisclosed and differs across vendors. Directional only.
E2	APAC smart healthcare USD 102bn (2026) to USD 603bn (2035), CAGR 21.8%	Towards Healthcare, APAC Smart Healthcare Market https://www.towardshealthcare.com/outlook/apac-smart-healthcare-market	n/c	Commercial market research	Low	Vendor forecast, method undisclosed. A different page from the same vendor showed an apparent typo, so its numbers need checking.
E3	APAC home healthcare USD 105.7bn (2025) to USD 343bn (2035), CAGR 12.5%	Precedence Research, APAC Home Healthcare Market https://www.precedenceresearch.com/databook/asia-pacific-home-healthcare-market	n/c	Commercial market research	Low	Vendor forecast, method undisclosed. Directional only.
E4	Global health-industries deal value +46% and volume -5% in 2025; APAC volume +12% (largest regional rise); China deals +53%	PwC, Global M&A trends in health industries: 2026 outlook https://www.pwc.com/gx/en/services/deals/trends/2026/health-industries.html	2026 outlook (day n/c)	Advisory firm deal analysis	Medium-High	Disclosed deal-data approach and well-known publisher. PwC is the engagement firm, so this is not independent. Claim-level attribution within the search summary is not confirmed.
E5	APAC about 16% of global healthcare PE deal value in 2024-25; Greater China healthcare PE more than doubled in 2025	Bain & Company, Global Healthcare Private Equity Report 2026 https://www.bain.com/insights/topics/global-healthcare-private-equity-report/	2026 (day n/c)	Advisory firm sector report	Medium-High	Reputable annual report with a disclosed deal-data basis. Read via search summary only.
E6	APAC H1 2026: PE buyouts strengthened and healthcare still produced significant transactions	Datasite, Deal Drivers APAC HY 2026 https://www.datasite.com/en/resources/insights/deal-drivers-apac-hy-2026	H1 2026 (day n/c)	Deal-platform data report	Medium	Based on the vendor's own platform data, so it may under-sample deals. Qualitative statement only.
E7	Asia medical trend rate 12.5% for 2026 (sixth straight double-digit year); Indonesia 17.8%, Philippines 16%, India 9.9%; 268 insurers surveyed Jun-Jul 2025	Mercer Marsh Benefits, Health Trends 2026 (Asia) https://www.marsh.com/content/marsh2/asia/my/en_my/services/employee-health-benefits/insights/health-trends-report.html	2026 edition; survey Jun-Jul 2025	Large insurer survey	Medium-High	Large sample and disclosed method. It measures insurer projections of benefit-plan cost, not hospital input costs, so it shows the direction of cost pressure rather than the target's own costs.
E8	Aon expects APAC medical cost growth to slow from 11.3% (2026) to 9.7% (2027)	Insurance Business Asia, reporting Aon https://www.insurancebusinessmag.com/asia/news/life-insurance/apac-medical-cost-growth-to-slow-significantly-in-2027-593200.aspx	2026 (day n/c)	Trade press reporting a broker forecast	Medium	Secondary reporting of a broker forecast. Useful as a cross-check on E7, not as a standalone source.
E9	APAC device regulation is tightening in 2026 (risk classification, labelling, AI); registration remains country-by-country	Lexology Pro, Three ways APAC medical device regulations are evolving in 2026 https://www.lexology.com/pro/content/three-ways-apac-medical-device-regulations-are-evolving-in-2026	2026 (day n/c)	Legal commentary	Medium	Legal commentary behind a paywall. Only the headline was visible.
E10	China and Japan device approvals typically 12-24 months; local-language submissions needed in several markets; reliance pathways expanding but not removing local requirements	MedDeviceGuide, APAC Medical Device Registration Strategy (2026) https://meddeviceguide.com/blog/asia-pacific-medical-device-registration-strategy-guide	2026 (day n/c)	Regulatory-consulting vendor blog	Low	Vendor marketing content; attribution of the timelines to this page rather than other results is not confirmed. Check against NMPA, PMDA, MFDS and HSA guidance before relying on any timeline.
E11	China VBP: average price of 10 selected coronary stents fell 92.67% (Tianjin interrupted time-series study)	PMC (peer-reviewed), Impact of VBP on coronary stent use in Tianjin https://pmc.ncbi.nlm.nih.gov/articles/PMC12581602/	n/c	Peer-reviewed study	High	Peer-reviewed with a defined method. It covers one city and one product category, so do not extrapolate to all devices.

ID	Claim	Source and URL	Source date	Type	Credibility	Rationale
E12	China VBP is a permanent structural feature of the device market; hospitals are guaranteed volume in return for steep price cuts	Regulatory Rapporteur, China's VBP revolution https://www.regulatoryrapporteur.org/medical-devices/chinas-volume-based-procurement-revolution-how-policy-procurement-and-industrial-strategy-are-reshaping-medical-device-market-access/1162.article	n/c	Regulatory professional journal	Medium	Professional publication. The 'permanent' conclusion is the author's judgement.
E13	81% of APAC respondents use generative AI; 67% cite cost as the main barrier; only 35% have dedicated AI teams	HIMSS AI Landscape APAC 2026, as reported by DistilINFO https://distilinfo.com/2026/08/24/himss-ai-landscape-apac-2026/	24 Aug 2026	Industry-body survey, secondary report	Medium	HIMSS is credible, but sample and respondent mix are unseen and the data were read via a secondary article. Respondents may be self-selected, so adoption may be overstated.
E14	Data quality, interoperability and governance gate AI success; 75% of providers expect larger gains from agentic AI than from GenAI alone	IDC, Asia/Pacific Healthcare in 2026 and Beyond https://www.idc.com/resource-center/blog/asia-pacific-healthcare-futurescape-2026/	2026 (day n/c)	Analyst-firm blog	Medium-High	Established analyst with its own surveys. Claim-level attribution of the 75% figure is not confirmed.
E15	Incompatible EHR platforms and differing privacy rules complicate cross-border health data sharing in APAC	Healthcare IT News Asia, What's next for APAC digital health in 2026 https://www.healthcareitnews.com/news/asia/whats-next-apac-digital-health-2026	2026 (day n/c)	HIMSS-owned trade press	Medium	Trade-press analysis, not primary data.
E16	Apollo Hospitals FY25 revenue Rs 21,794 crore, 73 hospitals, 8,000+ beds; Fortis FY25 revenue Rs 7,783 crore	INDmoney, Hospital stock peer analysis https://www.indmoney.com/blog/stocks/hospital-stock-peer-analysis	FY25 data; page date n/c	Retail-investor aggregator	Low	Aggregator quoting company results. Replace with the companies' filed annual reports.
E17	Ramsay Health Care group revenue A$17.8bn (2025); IHH operates 89 hospitals in 10 countries incl. Fortis stake	The Economy, Top 30 Private Hospital Groups 2026 / Ramsay wiki https://rankings.economy.ac/healthcare/news/2026/05/202605288858	May 2026 (from URL)	Ranking aggregator / wiki	Low	Secondary ranking and wiki content. Replace with filings.
E18	Diligence that misses sector-specific costs can destroy value (example: over USD 30m unexpected annual cost from missed compensation harmonisation)	EY-Parthenon, Transformational M&A drives healthcare value and growth https://www.ey.com/en_us/insights/strategy/transformational-m-and-a-drives-healthcare-value-and-growth	n/c	Advisory firm analysis	Medium-High	Reputable firm. The anecdote is one illustrative case, not a base rate. Read via summary only.
E19	Headline claims that 70-90% of M&A deals fail	AcquisitionStars, M&A Failure Rate https://acquisitionstars.com/ma-failure-rate	n/c	Practitioner blog	Low	Failure is defined inconsistently and the figures are recycled from other sources. NOT used as evidence in this memo. Listed so the committee knows the number is circulating and is weak.
E20	India has the highest APAC healthcare deal count; hospital roll-ups and diagnostics consolidation are active; JV structures common where foreign ownership is restricted	Arches Global, PE Healthcare Industry Trends 2026 in Asia https://arches-global.com/news/healthcare-industry-trends-asia-2026/	2026 (day n/c)	PE advisory blog	Low-Medium	Practitioner view without disclosed data. Needs corroboration from deal databases and local counsel.